Introduction

Heudia was formed as a result of a Phase 2 Small Business Innovation Research (SBIR) Contract with the U.S. Department of Agriculture to address the problem of Childhood Obesity, leading to the commercialization of AccessMeCare™. Our digital health platform, AccessMeCare™, is designed to connect healthcare providers, community organizations, and patients through an interoperable, cloud-based ecosystem.

By addressing barriers to care, streamlining data exchange, and empowering vulnerable populations, AccessMeCare™ aligns with the CMS Health Technology Ecosystem RFI's goals of enhancing interoperability, leveraging emerging technologies, promoting health equity, improving data transparency, and fostering stakeholder collaboration. This response outlines how AccessMeCare™ addresses the RFI's key questions and supports CMS's vision for a modernized, patient-centered digital health infrastructure.

Since it's commercial release, AccessMeCare™ has been deployed in over seven federally funded research initiatives and four existing sites with more than 250 clinical and non-clinical network partners, and over 50,000 direct user encounters. AccessMeCare™ is a Health Technology leveraging advanced machine learning, including graph theory, pathfinding algorithms, and predictive analytics, to streamline and enhance patient journeys by addressing clinical and community-level factors that adversely affect patient care transitions.

Heudia Health leverages advanced AI technologies to enhance care access, particularly for underserved populations. The team develops platforms that address access and availability of care by identifying and mitigating barriers such as geographic and socioeconomic challenges. Heudia excels in integrating multimodal data, including voice, imagery, and electronic health records (EHRs), to create comprehensive patient profiles. This enables precise population health management and reduces disparities in care delivery. The team's commitment to ethical AI is evident in bias mitigation strategies, use of diverse training datasets, and explainable AI frameworks to ensure fairness and transparency.

The team's leadership expertise in scaling technologies and fostering partnerships to deploy innovative solutions in complex healthcare environments. Our team comprises seasoned professionals with extensive experience in AI, healthcare, health system operations, geospatial technologies, and strategic development, uniquely positioning us to execute this project with excellence.

Our response incorporates the collective wisdom we have gathered from all of our ecosystem partners throughout this journey with key input from HSR.health. HSR.health is a leader in geospatial health informatics, leveraging location-based data to address public health challenges and improve healthcare outcomes by integrating environmental, socioeconomic, and healthcare access data to inform decision-making and optimize care delivery.

Basis for Our Response

Our response to this RFI is predicated on a long history of related work which started from a Phase 2 Small Business Innovation Research contract with the Department of Agriculture which included a Community Interoperability and Health Information Exchange (CIHIE) project funded by the Office of the National Coordinator for Health IT (Cooperative Agreement No. 90II0001/01-00) that grew to support several federal, state, and local initiatives spread across where we deployed Health Technology to create an ecosystem of over 250 multi-sector network partners serving over 50,000 individuals.

Priority Program Areas for Federal Collaboration and Innovation

Our experience includes work focused on frontier, rural, suburban, and urban environments. The Company was featured as one of eight Companies hosted by the U.S. Department of Health and Human Services (HHS) at the Hubert H. Humphrey Building in Washington, D.C., in 2018 to mark the department's inaugural effort to engage directly with startups and foster innovation within the federal health sector.

1. **Food Security and Nutrition Access**: Initiatives aimed at reducing food insecurity and improving nutritional outcomes for vulnerable populations.

2. **Mobile Integrated Health & Community Paramedicine:** Programs that extend care beyond hospital walls, leveraging EMS and community paramedics to provide in-home and preventative services.

3. **Healthy Start: Maternal and Child Health:** Federally funded programs focused on reducing infant mortality and improving perinatal outcomes among high-risk populations.

4. **Rural Health Network Development Planning:** Strategic planning efforts that foster collaboration among rural health providers to improve service integration and access.

5. **Rural Communities Opioid Response Program (RCORP):** Multi-sector interventions supporting treatment, prevention, and recovery from substance use disorders in rural areas.

6. **Rural Health Care Access:** Programs and policies focused on addressing systemic barriers to care in rural and frontier communities.

7. **Substance Use Disorder (SUD) / Opioid Use Disorder (OUD) Initiatives:** Comprehensive response frameworks supporting prevention, treatment, recovery, and harm reduction.

8. **Sequential Intercept Model Implementation:** Cross-agency strategies to divert individuals with behavioral health needs from the justice system into appropriate care pathways.

9. **Certified Community Behavioral Health Clinics (CCBHCs):** Federally supported, evidence-based clinics designed to expand access to comprehensive mental health and SUD services.

10. **Mental and Behavioral Health Services Expansion:** Programs addressing gaps in mental health service delivery, workforce shortages, and integration with primary care.

11. **Rural Health Infrastructure and Innovation:** Investments and initiatives to modernize health delivery systems in rural America through innovation, partnerships, and technology.

12. **Connected Care Pilot Initiatives:** Technology-enabled models designed to advance telehealth and remote patient monitoring for underserved and rural populations.

1. System Interoperability

RFI Question: How can CMS integrate systems like the National Provider Identifier (NPI), Provider Enrollment, Chain, and Ownership System (PECOS), and Medicare Care Compare with external data sources (e.g., state licensing boards, wearable devices) to streamline provider data and support research? What data elements are critical for a centralized health technology ecosystem?

Heudia's Response: AccessMeCare™ is a scalable PaaS that facilitates seamless data exchange across disparate healthcare and social service systems, addressing CMS's interoperability goals. The platform integrates independent programs, schools, universities, and community organizations into a unified network, enabling real-time data sharing. Key features include:

- Standards-Based Integration: AccessMeCare™ supports Health Level Seven (HL7) standards and Fast Healthcare Interoperability Resources (FHIR) APIs, ensuring compatibility with CMS systems like NPI, PECOS, and Medicare Care Compare. This allows for accurate provider data validation and verification, critical for a potential National Directory of Healthcare Providers & Services (NDH).
- Data Elements: The platform captures essential data elements, including provider credentials (NPI, licensing information), digital endpoints (secure messaging addresses), service locations, and SDoH-related data (e.g., housing, food security). These align with CMS's need for comprehensive provider and patient data to support care coordination and research, such as the NIH-CMS autism data platform.
- External Data Sources: AccessMeCare™ integrates with state licensing boards and community-based organizations, pulling verified data into a centralized hub. For example, it can connect with social service providers to share SDoH data, enhancing CMS's ability to address environmental stressors like poverty, as highlighted in studies on maternal and infant health outcomes.
- Verification Processes: The platform employs automated validation checks to ensure data accuracy (e.g., confirming NPI against NPPES) and supports secure endpoint verification via encrypted messaging, meeting CMS's requirements for data integrity.

The inclusion of HSR.health's GeoMD Platform provides spatial data infrastructure with the intent of merging disparate data from multiple separate repositories into a single framework to support AI/ML-based data analytics. Their pipelines can curate data from the various sources mentioned in the RFI as well as additional data from authoritative federal (e.g., Census, CMS, health outcomes data) as well as state and local sources of social, environmental, and health outcomes data, wearables data, and other data sources as may be needed to support health risk analytics the HHS and its agencies may deem important for protecting public health.

Further, Anna, HSR.health's generative AI orchestrator can provide this data lake with a conversational API that accepts natural language queries and returns explainable results. For example, this can enable HHS and CMS to perform research queries such as "Which Medicare cardiologists practice in heat risk areas?

Anna and the GeoMD Platform integrating disparate data elements, enable GeoAI analytic capabilities, along with an conversational layer allowing CMS to turn siloed provider files into a living, research-ready health-technology ecosystem.

By providing a unified interface for data exchange, AccessMeCare™ reduces administrative burden and supports CMS's goal of streamlining provider data across systems, enabling real-time access for providers, payers, and researchers.

2. Technology Integration

RFI Question: What role can AI, APIs, and wearable technologies play in improving care delivery, prior authorization, and research into chronic diseases? How can CMS mitigate risks like algorithmic bias or privacy breaches?

Heudia's Response: AccessMeCare™ leverages advanced technologies to enhance care delivery and support CMS's research objectives:

- APIs: The platform uses FHIR-based APIs to enable secure, real-time data exchange between healthcare providers, social services, and patients. For example, APIs facilitate remote intake and appointment scheduling, streamlining prior authorization processes by connecting payers and providers directly.
- AI and Behavioral Nudges: AccessMeCare™ employs AI-driven algorithms to guide care seekers to appropriate services based on their needs, incorporating behavioral nudges (e.g., reminders, personalized recommendations) to improve engagement. This supports CMS's focus on patient-centered care and chronic disease management.
- Wearable Technologies: The platform can integrate data from wearable devices (e.g., glucose monitors, fitness trackers) to provide real-time health insights, supporting research into chronic conditions like diabetes or autism. For instance, wearable data can inform the NIH-CMS autism data platform by tracking behavioral patterns or environmental stressors.
- Risk Mitigation: To address algorithmic bias, Heudia implements transparent AI models with regular audits to ensure equitable outcomes, particularly for underserved populations. Privacy is safeguarded through HIPAA-compliant encryption, role-based access controls, and secure cloud infrastructure hosted on Amazon Web Services (AWS). Penetration testing and compliance with NIST 800-53 standards further protect against breaches.

AccessMeCare™'s technology stack aligns with CMS's vision for innovative tools that enhance care coordination and research while prioritizing security and fairness.

This allows us to integrate 3rd party solution providers like HSR.health into our technology-driven ecosystem to provide chronic disease care ecosystem partners can act on, not just study.

As an examples, Anna can pulls daily streams from connected medical devices wearables, e.g., heart-rate patches, step counters, etc., and fuses them with GeoMD risk layers such as social and environmental factors, care desert maps, and other risk factors. Anna can then push a "watch list" into an EHR identifying those patients/ beneficiaries that are most likely to experience an adverse health event in the near term and why. Every alert ships with an explainable feature list, and each model retrain runs through a parity audit to confirm the same accuracy across race, sex, and dual-eligible groups.

This strategy also ensures that prior authorization is fair and nearly friction-free.

Anna's health risk stratification, when informed by diverse and patient-level data, can allow for Low risk, guideline-matched orders to be auto-approved in seconds, while high risk or edge cases can route to human reviewers with a plain language rationale attached. This can lower the overall burden of prior authorization to both providers and payers. However, and prior authorization is primarily a money-making scheme on behalf of payers, HHS is advised to remedy this matter through regulatory actions on insurers.

Anna orchestrates the device feeds, our GeoMD Platform enables patient/beneficiary risk stratification with explainable outputs offering CMS and payers a low risk path to AI-powered care, authorization, and research.

3. Health Equity

RFI Question: What strategies can ensure equitable access to health technologies for marginalized populations? How can CMS address barriers like digital literacy and broadband access?

Heudia's Response: AccessMeCare™ is designed to empower vulnerable populations, directly addressing CMS's health equity goals:

- Mobile Accessibility: The platform is accessible via a smartphone or tablet app, enabling low-income and rural populations to connect with care providers and social services without requiring advanced hardware. This reduces barriers for communities with limited broadband access.
- SDoH Focus: AccessMeCare™ integrates SDoH data to guide users to affordable care and community resources (e.g., food banks, housing programs). For example, it has supported maternal-fetal health programs, securing federal funding for six of eight clients to address poverty-related stressors, as evidenced by studies showing the impact of environmental factors on infant brain development.
- Digital Literacy: The platform features a user-friendly interface with multilingual support and guided workflows to accommodate users with low digital literacy. Community health workers can also use AccessMeCare™ to assist patients, bridging literacy gaps.
- Early Childhood Education: By connecting children to early education programs, AccessMeCare™ promotes long-term health equity, as education correlates with better health outcomes and reduced inequities.

These features ensure that marginalized populations, including rural, low-income, and minority communities, can access CMS-supported services equitably, aligning with the RFI's emphasis on inclusivity.

The strength of AccessMeCare's™ ecosystem architecture scales very rapidly to close care gaps across rural, frontier, and tribal communities where Anna and the GeoMD Platform can show and allow users to explore where physical and digital access gaps exist. Combining both the lack of brick-and-mortar medical facilities as well as the infrastructure (broadband, computing resources) required to support telehealth and digital health services along with data on chronic disease burden, can help target resource allocation to where they will have the greatest overall impact on improving health outcomes.

For example, because the Care Desert Initiative includes income, age, as well as broadband and infrastructure factors, CMS could pilot "device-plus-data" bundles (tablets + hot-spots) the same way it reimburses durable medical equipment.

4. Data Transparency

RFI Question: How can CMS enhance public access to Medicare Advantage data, provider directories, and quality metrics to empower beneficiaries and reduce fraud?

Heudia's Response: AccessMeCare™ enhances data transparency and supports CMS's transparency goals:

- Provider Directories: The platform maintains accurate, up-to-date provider directories by integrating with NPI, PECOS, and state licensing data, reducing errors and ensuring beneficiaries can locate appropriate providers.
- Quality Metrics: AccessMeCare™ enables providers to share quality metrics (e.g., patient satisfaction, readmission rates) through its dashboard, which can be made publicly accessible via CMS's Medicare Care Compare. This empowers beneficiaries to make informed decisions.
- Medicare Advantage Data: By connecting payers and providers, AccessMeCare™ facilitates the sharing of Medicare Advantage (MA) encounter data, supporting CMS's goal of increasing MA data transparency for public analysis and fraud detection.
- Public Access: The platform's cloud-based architecture allows CMS to expose de-identified data sets to researchers and the public, supporting initiatives like the NIH-CMS autism data platform while maintaining HIPAA compliance.

By centralizing and validating data, AccessMeCare™ reduces fraud risks (e.g., ghost providers) and enhances beneficiary access to transparent information, aligning with CMS's objectives.

Through our partnership with HSR.health we can turn raw files into a public, map-based front door. The GeoMD Platform geocodes every provider and plan record and layers it on an interactive map that any beneficiary can filter by specialty, network status, star rating, travel time, and a host of other factors of interest. Further, Anna allows a conversational interface to this data meaning no spreadsheets or PDFs are needed, and more information is more readily accessible to all.

The GeoMD Platform and Anna offers a novel means of analyzing claims: geospatially. A provider billing for home visits 300 miles apart on the same day can be flagged; a clinic suddenly performing telehealth visits at 3 AM across time zones triggers an alert. These signals feed CMS fraud detection teams in near real time. Further, the algorithm's ability to detect fraud from simply unusual but legitimate activity will improve over time.

The public map accepts user-reported errors ("This office moved") that flow back through Anna for verification — advancing the update cycle faster than annual plan submissions.

The result - patients, beneficiaries, and researchers get a live, visual directory they can trust, app developers gain a clean API, and CMS gain geospatial fraud and data quality safeguards without adding bureaucratic steps.

5. Stakeholder Collaboration

RFI Question: How can CMS partner with state agencies, private entities, or initiatives like the Office of the National Coordinator for Health Information Technology (ONC) to build a robust ecosystem?

Heudia's Response: Heudia's collaborative approach positions AccessMeCare™ as a key partner for CMS:

- State and Local Partnerships: AccessMeCare™ has integrated with state agencies and regional organizations, such as the Rappahannock Area EMS Council and RHP-NWAHEC, to deliver coordinated care. These partnerships can support CMS's integration with state licensing boards and Medicaid programs.
- Private Sector: Heudia's work with managed care organizations, hospitals, and community-based organizations demonstrates its ability to bridge public and private sectors, aligning with CMS's goal of multi-stakeholder collaboration.
- ONC Alignment: AccessMeCare™ adheres to ONC's interoperability standards (e.g., FHIR, USCDI), ensuring compatibility with ONC's Trusted Exchange Framework and Common Agreement (TEFCA). This enables seamless data exchange with ONC-certified systems, supporting CMS's interoperability efforts.
- Federal Funding Success: Heudia's track record of securing federal funding for SDoH programs (e.g., maternal health initiatives) positions it as a reliable partner for CMS to implement grant-funded technology projects.

AccessMeCare™'s interoperable, partnership-driven model fosters the collaborative ecosystem CMS envisions, connecting federal, state, and private stakeholders to enhance care delivery.

Challenges and Mitigation Strategies

While AccessMeCare™ addresses the RFI's objectives, potential challenges include:

- Scalability: Integrating with legacy CMS systems (e.g., PECOS) may require custom adapters. Heudia mitigates this by offering modular APIs and dedicated integration support.
- Adoption: Providers may resist new platforms due to workflow changes. Heudia addresses this through user training, intuitive design, and demonstrated ROI (e.g., reduced costs for hospitals and payers).
- Data Privacy: Sharing SDoH and wearable data raises privacy concerns. AccessMeCare™'s HIPAA-compliant infrastructure and consent management tools ensure compliance and user trust.
- Equity Gaps: Some rural areas lack smartphone access. Heudia partners with community organizations to provide device access and in-person support.

These strategies ensure AccessMeCare™ can scale to meet CMS's needs while addressing implementation barriers.

The GeoMD Platform itself supports Heudia's ecosystem by adding a new form of health risk analytics that has been demonstrated in numerous federally-funded (NASA, NOAA, USGS, FEMA, DHS, and NGA) pilots.

This holistic approach allows us to offer a spatial data infrastructure using Anna as a conversational, generative AI interface producing explainable responses, CMS can turn disparate federal, state, local, tribal, as well as private authoritative datasets into one robust ecosystem—without reinventing infrastructure or compromising privacy.

For example by integrating HSR.health's solution set into the AccessMeCare™ ecosystem, we can:

1. Provide the common geospatial backbone that everyone can plug into. The GeoMD Platform already aligns federal (e.g., CMS, CDC), state, and private datasets to the same Census-tract geometry. By letting ONC's TEFCA networks and state HIEs publish FHIR feeds with a GeoMD tract tag, CMS can give every partner an instant way to merge claims, EHR, SDOH, and environmental data without designing a new data model.
2. Use Anna as a neutral, explainable query layer across agencies. CMS, state Medicaid programs, and private plans often store data in separate clouds. Anna, as a generative AI orchestrator, can sit above those silos, route a plain-language question ("Show diabetic readmissions in wildfire-risk tracts for 2024") to each source, then fuse the answer with a full audit trail. ONC can certify Anna as an "AI edge service," so any TEFCA participant can invoke it without moving PHI to a third party.
3. Create shared "risk indices" that state and federal programs can adopt. The GeoMD Platform includes numerous relevant health risk datasets, indicators, and indices that can e made available to all partners. This includes our Heat-Health Risk Index piloted in Manhattan (funded by DHS, NOAA, NGA), Opioid Abuse Related Mortality risk index, our Disaster Health Risk Indices (NASA, NOAA, USGS, FEMA) and others are already available for use today. CMS can co-brand these indices and push them through ONC's USCDI+ public-health domains so every state Medicaid agency along with private ACOs can stratify members with the same methodology to ensure consistency across American healthcare.
4. Run joint pilots that pair federal incentives with state boots on the ground. For example, CMS could fund a pilot in select (e.g., 3 states) where CAP civil air patrol imagery flows into the GeoMD Platform for post-disaster hospital infrastructure assessment; state EM agencies supply the fly-over, HSR.*health* runs the AI change detection algorithm, CMS evaluates how fast hospitals reopen. Results can be shared broadly across the country for replication and/or follow-on pilot efforts.
5. Offer a "plug-and-play" API gateway for private innovators. - Through HSR.*health*'s APIs, app developers can pull de-identified GeoMD Platform layers and push analytics back to CMS quality dashboards. CMS can require any CMMI pilot or ACO REACH participant that uses social, environmental, and health outcomes data to surface their derived metrics via the same gateway, creating a virtuous data loop instead of new silos.

What is Proposed – MERIT – A National Demonstration of AI-Powered Health Technology Ecosystem Supporting Secretary Kennedy's Vision to "Make America Healthy Again"

Project Overview

Heudia Health proposes a large-scale, national bipartisan demonstration of *MERIT*—the Model Evaluation & Real-time Integration Tool—delivered through its *AccessMeCare™* platform. MERIT is a real-time performance monitoring and self-correction framework that ensures predictive AI models remain transparent, equitable, and optimized across diverse populations and care settings. The platform directly supports CMS's RFI priorities to enhance interoperability, reduce health disparities, and foster patient-centered innovation.

Demonstration Objectives

This proposed demonstration will quantify MERIT's impact on:

- Reducing per capita healthcare expenses associated with uninsured and publicly insured individuals over utilization of avoidable emergency department use and preventable hospital admissions/readmissions;
- Improving uptake of primary and preventive care in Medicaid and Medicare populations;
- Expanding equitable access to care across geographies and populations.

Resources Required

To successfully implement this initiative under a federal grant or IDIQ contract, Heudia Health will require:

- **Federal Support:** Multi-year funding (3–5 years) to support platform expansion, data integration, and model validation in partnership with CMS-regulated entities;
- **Community Sites:** Deployment across a minimum of 3-5 high-need regions associated with approximately the same number or more congressional districts regions with support from local hospitals, FQHCs, EMS agencies, and community-based organizations;
- **Stakeholder Engagement:** A national Community Advisory Board and bipartisan coalition of federal, state, and local health care leaders to guide implementation and policy alignment.

Technology Alignment

AccessMeCare™ already meets HHS interoperability standards (FHIR, HL7), supports ONC's TEFCA framework, and is hosted on HIPAA- and NIST 800-53-compliant infrastructure. MERIT enhances these capabilities with AI tools that detect model degradation, correct in real time, and map risk at the population level.

Bipartisan Vision, Real-World Results

This project directly supports Secretary Kennedy's call for scalable, evidence-based innovation that bridges political divides through measurable improvements in cost, access, and quality. MERIT is built for bipartisan success: focused on measurable ROI for payers and providers and ecosystem accountability in how AccessMeCare™ shapes care delivery.

The Challenge

Billions in preventable healthcare costs stem from fragmented care transitions, insufficient real-time risk insights, and technology that fails to serve the full diversity of patients—particularly in rural, low-income, and underserved communities. MERIT addresses these failures by providing a real-time, adaptive intelligence layer that continuously improves care navigation for individuals most at risk.

Conclusion

Heudia's AccessMeCare™ platform is uniquely positioned to advance CMS's Health Technology Ecosystem vision. By enabling interoperability, leveraging AI and APIs, promoting health equity, enhancing data transparency, and fostering stakeholder collaboration, AccessMeCare™ addresses the RFI's core objectives. Its HIPAA-compliant, cloud-based architecture, proven success in SDoH programs, and alignment with ONC standards make it a scalable solution for CMS's 150 million beneficiaries.

Heudia welcomes the opportunity to partner with CMS to build a patient-centered, equitable, and innovative health technology ecosystem, supporting Secretary Kennedy's "Make America Healthy Again" agenda and improving outcomes for Medicare, Medicaid, CHIP, and Marketplace beneficiaries.

Through this demonstration, Heudia Health will show what's possible when predictive technology is applied to create, manage, and actively monitor a self-sustaining Health Technology Ecosystem.

We invite CMS and HHS to partner with us—through a grant or IDIQ mechanism—to scale this breakthrough solution and deliver lasting change for over 150 million Americans who rely on publicly supported health coverage.

For More Information Reach Out To:

Ed Connors, CEO Heudia
908-265-6282
edconnors@heudia.com